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# BRN Case Management CE Package - Draft / Pending BRN CEP Approval

BRN Care Manager / RN Case Management CEP | Draft / Pending BRN CEP Approval

## BRN Case Management CE Package - Draft / Pending BRN CEP Approval

### Chronic Disease, Geriatric, and High-Risk Patient Management

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"Pending California Board of Registered Nursing Continuing Education Provider approval. Do not advertise, issue certificates, or represent this course as approved until a BRN CEP number has been issued."

*This document follows a CCCCCO-style documentation organization only (structured overview, objectives, content outline, activities, facilitator notes, assessment mapping, compliance support). No CCCCCO content is copied; all content is original and customized for California BRN Case Management CE.*

### Module At a Glance

| Field                     | Value                                                                                                               |
|---------------------------|---------------------------------------------------------------------------------------------------------------------|
| Module ID                 | CM-M09                                                                                                              |
| Module title              | Chronic Disease, Geriatric, and High-Risk Patient Management                                                        |
| Parent course             | RN Case Management in Home Health: Care Coordination, Documentation, Patient Advocacy, and Safe Transitions of Care |
| Contact hours             | 3 (draft design math)                                                                                               |
| Estimated learner minutes | 170 (>=150 floor)                                                                                                   |
| Direct/indirect linkage   | Direct patient/client care (scientific knowledge)                                                                   |
| Approval status           | Draft / Pending BRN CEP Approval                                                                                    |

### BRN Relevance Rationale

Applies clinical/scientific knowledge to manage high-risk populations and reduce decompensation.

### Module Description

Strengthens risk-stratification, chronic-disease management priorities, geriatric-syndrome awareness, and monitoring plans with escalation triggers. Educational content only; not medical advice.

### Measurable Learning Objectives

By the end of this module the learner will be able to:

- 1 Apply risk-stratification to identify high-risk patients in a panel.
- 2 Describe nursing management priorities for common chronic conditions in care management.
- 3 Identify geriatric-specific risks (falls, polypharmacy, cognition) and nursing mitigations.
- 4 Build a monitoring plan with escalation triggers.

### Required Readings / Learning Content Sections

- Reading 9.1 - Risk stratification.
- Reading 9.2 - Chronic-disease management priorities.
- Reading 9.3 - Geriatric syndromes and polypharmacy.
- Reading 9.4 - Monitoring plans and escalation triggers.

### Detailed Lesson Outline

| Lesson ID | Lesson                                          | Est. minutes |
|-----------|-------------------------------------------------|--------------|
| CM-M09-L1 | Risk stratification                             | 40           |
| CM-M09-L2 | Chronic-disease management priorities           | 40           |
| CM-M09-L3 | Geriatric syndromes & polypharmacy              | 35           |
| CM-M09-L4 | Monitoring plans & triggers                     | 35           |
| CM-M09-KC | Knowledge check + risk-stratification worksheet | 20           |

Total estimated instructional minutes: **170** (meets the  $\geq 150$ -minute / 3-contact-hour floor).

### Key Terms

| Term                | Definition                                                           |
|---------------------|----------------------------------------------------------------------|
| Risk stratification | Classifying patients by risk to target resources.                    |
| Polypharmacy        | Use of multiple medications, raising interaction/adverse-event risk. |
| Geriatric syndrome  | Multifactorial conditions common in older adults (falls, delirium).  |
| Escalation trigger  | A predefined change that prompts a higher level of action.           |
| Decompensation      | Functional/clinical decline as a condition worsens.                  |

### Fictional / De-identified Case Scenario

*All clinical content is fictional and de-identified. No PHI.* A fictional 80-year-old with heart failure, diabetes, and mild cognitive impairment takes nine medications and had one recent fall.

### Learner Activities

- Read Sections 9.1-9.4.
- Stratify the client's risk level with justification.
- Identify polypharmacy and fall risks with nursing mitigations.
- Build a monitoring plan with  $\geq 3$  triggers and complete Knowledge Check 9.

### Documentation / Decision-Making Exercise

Complete a Risk-Stratification + Monitoring Worksheet for the scenario, including at least three escalation triggers tied to specific findings.

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## Knowledge Check Blueprint

10 items, unlimited attempts, immediate feedback, no full key,  $\geq 80\%$  to complete.

### Assessment Items (Learner-Facing Stems)

**KC9-Q1 (Multi-select).** Which factors raise this geriatric client's risk profile? (select all)

- A) Polypharmacy (9 meds)
- B) Recent fall
- C) Cognitive impairment
- D) A single stable condition

**KC9-Q2 (MCQ).** An appropriate escalation trigger in a heart-failure monitoring plan is:

- A) Weight gain of  $\geq 3$  lb in a day / 5 lb in a week
- B) Any change in TV channel
- C) Normal daily walk
- D) Stable weight

**KC9-Q3 (True/False).** Polypharmacy increases the risk of adverse drug events and warrants medication review.

- True
- False

### LMS Completion Criteria

- All lesson pages viewed (activity completion: require view).
- CM-M09 knowledge check passed at  $\geq 80\%$  (require passing grade).
- Documentation/decision exercise submitted (require submission).

### Evidence Generated by Learner Completion

- Activity-completion timestamps (time-on-task evidence).
- Knowledge-check score and attempt log.
- Submitted exercise artifact (retained  $\geq 4$  years; no PHI).

### Moodle Activity Recommendations

- Page
- Book
- Quiz (KC9)
- Assignment (worksheet)

### Accessibility Considerations

- Captions + transcripts for any media; semantic headings; alt text on diagrams.
- Sufficient color contrast; no color-only meaning; keyboard-navigable activities.
- Accessible downloadable templates (tagged PDF + DOCX); extended-time accommodation supported.
- Target WCAG 2.1 AA on the built course.

## PHI / Privacy Guardrails

- All cases fictional and de-identified; no real patient/resident/family/facility data.
- Fictional clinical profile only; avoid color-only risk tiers (use labels).
- Submission reminders and (for capstone) attestation + manual PHI review.

## SME Review Flags

- [[SME REVIEW REQUIRED]] Confirm clinical content currency; mark as education, not medical advice or standing orders.

## Compliance Review Flags

- [[COMPLIANCE REVIEW]] Fictional clinical profile only; avoid color-only risk tiers (use labels).

## Internal Answer Key Appendix (ADMIN / FACULTY ONLY - DO NOT SHOW LEARNERS)

| Item ID | Type         | Correct answer | Rationale                                                           | Remediation             |
|---------|--------------|----------------|---------------------------------------------------------------------|-------------------------|
| KC9-Q1  | Multi-select | A, B, C        | A-C are high-risk geriatric markers; D is comparatively lower risk. | Review Reading 9.1/9.3. |
| KC9-Q2  | MCQ          | A              | Rapid weight gain can signal fluid overload requiring escalation.   | Review Reading 9.4.     |
| KC9-Q3  | True/False   | True           | Multiple medications raise interaction/adverse-event risk.          | Review Reading 9.3.     |

## Debrief / Remediation Guidance

Show how stratification drives monitoring intensity and triggers, and how polypharmacy review prevents harm. Failed attempts route to Reading 9.1/9.3/9.4. Reinforce content is educational, not medical advice.

## Application-Packet Defensibility Notes

Application of scientific/clinical knowledge to high-risk patients is core direct patient care and strongly BRN-relevant.

**Program-structure note:** This module is part of the 30-contact-hour, 10-module RN case management program (10 modules x 3 contact hours; 50 instructional minutes = 1 contact hour; minimum 1,500 instructional minutes, 1,680 design minutes). The official BRN contact-hour map and transfer worksheet are aligned to this 30-contact-hour structure. Any earlier 4-contact-hour representative course is superseded and is not part of the current BRN CEP signer-review package. [[SME REVIEW REQUIRED BEFORE SUBMISSION]]

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